

**SHRI BALAJI KIDNEY CARE**

PVT. LTD

Visit No:	2025-26/918	Age / Sex:	24 / Male
Patient Name:	MAULIK GHANSHYAMBHAI THUMAR	Collected Date:	25-02-2026
Referred By :	DR. DHANANJAY	Received Date:	26-02-2026
Referred From :	CONTINENTAL HOSPITAL, HYDERABAD	Reported Date:	27-02-2026
Specimen:	Tissue		

Allograft Renal biopsy LM+C4D+SV40

Specimen: Allograft Kidney (needle) biopsy

H26- 280

Brief Clinical History:

24 years old male, k/c/o LRKT on 07/01/2026 : ABO Compatible , basic disease : anti GBM disease, previous transplant kidney biopsy on 21/01/2026, borderline changes suspicious of Acute TCMR, 2 doses of Solumedrol, urine c/s : 24/01/2026 E.coli, serum creatinine : 1.64 to 1.87 to 2.05 to 1.87 to 1.97mg/dl, Tac Level : 10.1, CUE : No Protein, Allograft dysfunction, ? Rejection.

Specimen: Allograft Kidney (needle) biopsy**Gross Pathology:**

1. Received , single linear gray white core, in 10% NBF, measuring 1.7cm in length. All tissue processed for light microscopy.

Microscopic Description:

Serial sections and stains (H&E, PAS, Methenamine Silver & Masson Trichrome) studied show a core of renal subcapsular cortex with adjoining part of medulla with up to 11 glomeruli and a small artery.

First Floor, Door No.1-99/14, Megha Hills, Opp. to Madhapur Metro Pillar No.1734,
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Glomeruli:

A single glomerulus is obsolescent. The viable glomeruli appear unremarkable with no evidence of significant increase in cellularity ,glomerulitis, or fibrin thrombi.

Tubules:

There is mild tubular injury with few loss of the brush border. Few tubules about show cytoplasmic blebbing. Tubular atrophy is seen over less than 05% of the cortex sampled. There is no evidence of significant mononuclear tubulitis seen in the biopsy studied.

Interstitial:

The interstitium is widened and fibrosed around the atrophic tubules with a mild patchy infiltrate of lymphocytes and occasional plasma cells; involving less than 10% of the non-fibrosed cortex.

Vessels:

Mild arteriolar hyalinosis is seen in occasional vessels. There is no evidence of significant peritubular capillaritis/ arteriosclerosis/endarteritis endarteritis seen in the biopsy studied.

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Immunohistochemistry:

IHC for C4d stain does shows focal irregular faint positivity around the walls of few peritubular capillaries, which is not significant to consider it as positive.

IHC for SV 40 stain is negative for Polyomavirus.

Diagnosis/Comments:

Allograft Kidney (needle) biopsy shows features of **Mild Acute Tubular Injury.**

There is no definite evidence of rejection seen in the biopsy studied.

Banff score: g0, t0, i0, v0, cg0, ct1, ci0, cv0, ptc0, c4d 0, mm0, aah1, ti0, i-IFTA 0.

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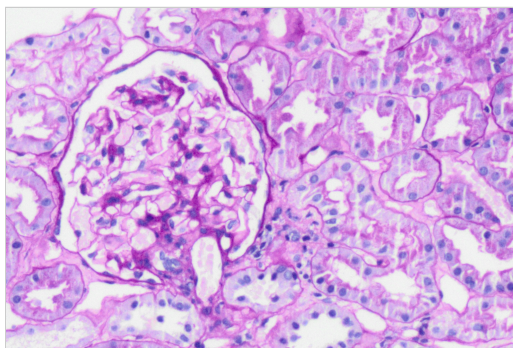
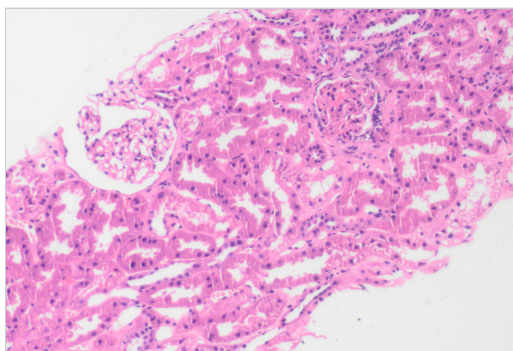


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Dr. RADHIKA K PATIL

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